

The aesthetics theme

An exploration. What it needs, and where it now sits.

Superseding note. This paper explored an aesthetics anchor taking the ground floor. That is not the configuration. **The ground floor is the Alameda / Medbury Conversion Clinic**, with imaging and phlebotomy as campus facility alongside it. The aesthetics theme therefore lives on the **first floor**, expressed through who is recruited into the plaza: the FUE suite in the only room large enough for it, the Playroom as the treatment room, and Ring 1 practitioners filling the eight consulting rooms. The clinical analysis in sections 02 to 07 stands unchanged and is the reason the theme works. The ground-floor space plan and the anchor economics in sections 08 and 09 do not apply.

01 / THE THESIS

Anchor first, then layer.

Themed on aesthetics and cosmetic practice, mirroring the Lagos arrangement, with The Cloister and the private consulting-room rentals layered on top. It is the right instinct and it is stronger than the generic medical park for four reasons.

Why the anchor thesis works	Reason
An identity, not an address	A park with an anchor product profile is sellable. A generic address has to be explained
The right patients, already in the building	Aesthetic patients are cash-pay, repeat, and precisely the demographic that buys a Cloister family membership. The layering is a genuine funnel, not co-location
Appeal to the other tenants	A specialist renting a room by the session is buying access to a patient base that is already on site for something else
Uniformity across Lagos and Abuja	One structure, one brand, one operating model, and a template for the third site. That is worth something on its own

Your requirement, taken literally, splits in two. The ambulatory side can satisfy **all office-based and local-anaesthesia day case work**, which is the large majority of aesthetic revenue and almost all of the repeat revenue. It cannot satisfy sedation or general-anaesthesia surgery inside this building, and that limit is clinical rather than financial. Sections 03 and 05 set out what that means and what it would take to change it.

02 / THE THREE TIERS

Aesthetic work sorted by what the facility must provide.

Anaesthesia and recovery decide the room. Everything else follows.

Tier	Anaesthesia	Recovery	Cases	Facility needed
Tier 1 Non-surgical	Topical or none	None to 15 minutes, ambulant	Botulinum toxin, dermal fillers, chemical peels, microneedling and RF microneedling, laser hair removal, pigmentation and non-ablative resurfacing, HIFU, RF tightening, cryolipolysis, PRP face and scalp, mesotherapy, skin boosters, IV therapy	Treatment room. First floor is fine
Tier 2 Minor surgery, local	Local infiltration	30 to 60 minutes, walks out	Mole and lesion excision, earlobe repair, scar revision, small lipoma, upper-lid blepharoplasty, labiaplasty, thread lifts, and FUE hair transplant	Clean procedure room. Ground floor for level access
Tier 3 Day case surgery	Sedation or general	2 to 6 hours, monitored	Liposuction beyond small volume, abdominoplasty, breast augmentation and reduction, rhinoplasty, facelift, gynaecomastia, fat transfer and buttock augmentation	Theatre with anaesthetic support. Not possible in this building

Tiers 1 and 2 together are typically **80% or more of an aesthetic clinic's case volume** and the overwhelming majority of its repeat revenue, because injectables and devices bring patients back every three to six months while surgery does not. Tier 3 is high ticket and low frequency.

03 / THE LIMIT

Why Tier 3 cannot happen here, at any budget.

The stopper is evacuation. A patient under sedation or general anaesthesia who deteriorates must be moved horizontally, on a trolley, to an ambulance. This building has no lift and the staircase is in the right-hand block. Stairs are not an option for an unconscious patient. That is a clinical governance failure no budget corrects, and it applies to the first floor absolutely.

Three further requirements compound it, all of which a converted residential house resists:

Requirement	Why the building resists it
Ventilation	Positive pressure with HEPA filtration and 15 to 25 air changes an hour. That is an air handling unit, ductwork and a plant room. The campus is specified on zoned split units precisely because none of those exist
Power	Isolated power supply panels and UPS-backed essential circuits. A different order of electrical work from the rest of the fit-out
Staffing and cover	Surgeon, anaesthetist, scrub and circulating nurse, recovery nurse, plus resuscitation capability and a written transfer agreement with a receiving hospital

What this does not prevent. A surgically oriented anchor can still be built here. Upper-lid blepharoplasty, labiaplasty, thread lifts, lipoma and lesion excision, scar revision and full FUE hair transplant are all Tier 2 and all deliverable in a ground-floor clean procedure room. The line is drawn at sedation, not at surgery.

04 / THE FUE INSIGHT

The anchor that fits this building better than any other.

Follicular unit extraction is worth singling out, because it is the rare high-ticket surgical service that suits a converted house with one reception and no lift.

Why it fits	Detail
Anaesthesia	Local only. No anaesthetist, no monitored recovery, no evacuation problem
Session length	Six to ten hours. One patient occupies the suite for the whole day
Effect on throughput	One arrival per day per suite. It relieves the single-reception constraint rather than worsening it
Ticket	NGN 1.2M to 4M in Nigeria by graft count. Take NGN 2.5M as an average
Room requirement	25 to 30 sqm: reclined chair, three to four operators working simultaneously, instrument trays, excellent task lighting
Patient at discharge	Awake, ambulant, walks out with a dressing

One FUE suite at two cases a week is about NGN 230M a year of clinical revenue. A sessional consulting room on the first floor earns NGN 28.6M. That is eight times the yield from the same floor area. It is also the least demanding use in the building on reception, waiting and parking, because it brings one patient a day instead of thirty.

This argues for pursuing the hair-restoration SPV as a **named anchor in its own right**, not as a service line inside a general aesthetics practice. It has different staffing, different room design, a different patient journey and a different marketing channel, and it justifies its own dedicated suite.

05 / IF TIER 3 IS WANTED

Three routes, and what each costs.

Route	What it means	NGN M	When
Partner a nearby hospital for theatre lists	The anchor consults, assesses and follows up at Lyfe Place, and operates at a partner theatre. Patient stays inside the Lyfe Place relationship throughout. No capital	nil	Phase 1
Modular theatre unit sited on the grounds	Solves ventilation, isolated power and level access in one, at ground level next to the aesthetics suite. This is the coherent long-term campus plan	90 to 150	Phase 2, on volume
Theatre inside the main building	Consumes three or four of the highest-yielding rooms and still fails the evacuation test	not viable	Never

Recommendation: open on the partner-theatre route. It costs nothing, it lets the anchor sell a full surgical offer from day one, and it defers a NGN 90M to 150M decision until there is a list volume to justify it. Write the transfer and theatre-access agreement before the anchor signs, because it is part of what they are buying. A modular unit becomes the obvious phase 2 move once the anchor is doing more than about two GA cases a week.

06 / SPECIALTIES

Two rings, and why the outer one is not a dilution.

Ring 1: the anchor and its immediate adjacencies

Ring 1	Why it belongs
Aesthetic and plastic surgery	The anchor. Injectables, devices, Tier 2 surgery, and Tier 3 through the partner theatre
Hair restoration, FUE	Its own anchor and its own suite. See section 04
Dermatology, medical and cosmetic	The clinical spine of the theme. Also brings skin cancer excision, which is Tier 2
Aesthetic gynaecology and intimate health	High margin, discreet, underserved in Abuja
Weight management and metabolic	The GLP-1 era makes this the fastest-growing adjacency in aesthetics anywhere
Longevity and hormone optimisation	Where Medlyfe fits when it comes in as a first-floor tenant
Dental aesthetics and smile design	Same patient, same decision, different clinician
Oculoplastics	Blepharoplasty under local. Bridges ophthalmology and aesthetics
Vascular, veins and sclerotherapy	Cosmetic and medical at once, and device heavy

Ring 2: aligned enough to let by the session

These are not filler. **Every one of them is something the surgical anchor needs and would otherwise have to send patients out for.** Letting rooms to them completes the theme rather than diluting it.

Ring 2	What the anchor needs it for
Cardiology	Pre-operative clearance for any sedation or general anaesthesia case
Endocrinology	Diabetes and thyroid control before surgery, and it feeds weight management
Anaesthetics	A pre-assessment clinic, even when the surgery itself happens elsewhere
Psychiatry and psychology	Body-image assessment and bariatric psychological clearance, which is a genuine requirement and not a courtesy
Physiotherapy	Post-operative rehabilitation and lymphatic drainage after liposuction
Nutrition and dietetics	Pre and post-surgical optimisation, and GLP-1 programmes
Obstetrics, gynaecology, fertility	Same demographic, ultrasound intensive, high willingness to pay
ENT	The rhinoplasty referral partner, plus sleep and snoring work

The test for admitting a specialty should be: **does it either serve the anchor's patients or draw the anchor's kind of patient into the building?** Anything that does neither belongs somewhere else, however good the practitioner.

07 / ROOM SPECIFICATIONS

What each room must be able to do.

Room	sqm	Where	Must be able to do
Treatment room x 2	16 to 20 each	First floor or ground	Tier 1. Procedure couch with trendelenburg, mobile task light, laser-safe blinds and door interlock, eye protection store, device trolleys, hand-wash, sharps and clinical waste, mirror and photography station with fixed lighting for before and after imaging
FUE suite x 1 to 2	25 to 30 each	Ground floor	Tier 2, all day. Reclined transplant chair, room for three to four operators, microscope or magnification station, graft storage on ice, two instrument trolleys, excellent shadow-free lighting, patient entertainment, adjacent WC access
Clean procedure room	28 with recovery	Ground floor only	Tier 2 surgical. Filtered supply at modest positive pressure from a dedicated unit, operating light, diathermy, piped or cylinder oxygen and suction, resuscitation trolley, scrub position, level trolley route to the ambulance bay, two recovery bays
Consultation room	14	Either floor	Assessment, consent, photography, quotation. Needs to feel like the practice, not a clinic room
Recovery lounge	20	Ground floor	Post-treatment observation for Tier 1 and 2. Recliners, refreshments, discreet exit
Device store	8	Ground floor	Secure. Aesthetic lasers and energy platforms are the most valuable movable assets on the campus

Two specification points that get missed. First, **laser safety**: door interlocks, controlled-access signage, laser-safe window treatments and eyewear storage are regulatory, not optional, and they are cheap at design stage and expensive later. Second, **the photography station**: standardised before-and-after imaging with fixed lighting and positioning is how aesthetic practices sell, defend outcomes and manage complaints. It belongs in the room specification, not in someone's phone.

08 / SPACE PLAN

The ground floor as an aesthetics and day-procedure anchor.

Ground floor	sqm
2 treatment rooms: injectables, lasers, energy devices	36
2 FUE hair transplant suites	52
Clean procedure room plus recovery bay, level access	30
Post-treatment recovery lounge	20
Consultation room	14
Phlebotomy draw point and medication hatch	14
Device store, secure	8
Ultrasound room	27
Total, of 201 usable	201

An aesthetics anchor removes the need for the 55 sqm X-ray and echocardiography suite. Aesthetic and Tier 2 surgical work needs bloods and ultrasound, not radiography. Diagnostics on this campus becomes a phlebotomy draw point, the laboratory in the guest chalet, and one ultrasound room. That releases about 40 sqm on the ground floor and cuts the diagnostics fit-out from NGN 75M to roughly NGN 45M.

The counterweight: the Ring 2 specialists on the first floor do want imaging, so the decision is whether to keep a reduced imaging capability for them or refer imaging out. That is a live question and it turns on how many Ring 2 rooms you expect to let.

09 / WHAT IT IS WORTH

And the ground floor now has two claimants.

Anchor clinical revenue, NGN M	Conservative	Mid	Mature
Tier 1, injectables and devices, 2 rooms	298	442	552
Tier 2, minor surgery	55	83	83
FUE suites	172	230	460
Anchor clinical revenue	526	754	1,095
Medbury's take, NGN M	Conservative	Mid	Mature
Licence, at 18% of anchor revenue	95	136	197
Product and skincare retail margin	47	68	99
Pre-operative bloods	8	8	8
Share of anchor SPV profit, 45% of 25%	59	85	123
Total to Medbury	209	297	427

Product retail is a line a general medical park does not have. Aesthetic practices sell skincare, post-procedure and maintenance products at 40% to 60% margin, typically 20% of clinic revenue. Medbury Pharmaceuticals should own that shelf, and it is worth NGN 47M to NGN 99M a year on its own.

The ground floor now has two claimants and they cannot both have it. The Alameda Conversion Clinic SPV is worth about NGN 168M a year to Medbury from that floor, being a NGN 132M licence plus roughly NGN 36M of capture. The aesthetics anchor is worth **NGN 209M to NGN 427M, or 1.2 to 2.5 times as much**, and the money is retained in Nigeria rather than landing in Cairo.

There is a resolution that does not require dropping Alameda. What Alameda actually needs is **consulting rooms for visiting specialists running screening clinics**, which is exactly what the first floor is. Move the Conversion Clinic upstairs as a large block sessional user, and the ground floor goes to the anchor. Alameda loses a dedicated floor and gains flexibility and a lower cost base, which is arguably an easier sell than the current NGN 132M for 104 sqm.

Where	Who	How Medbury earns
Ground floor	Aesthetics and day-procedure anchor, plus the hair-restoration SPV	Licence, SPV equity, product retail
First floor	The private medical plaza. Ring 2 specialists by the session, plus Alameda as a block user, plus Medlyfe when it comes	Membership and session fees
Guest chalet	Medbury Diagnostics laboratory	Facility, recovered in rates
Boys' quarters	Medbury Pharmaceuticals dispensary and the product shelf	Facility, plus retail margin

10 / WHAT TO DECIDE

Five questions, in order.

Question	What turns on it
1 Does the aesthetics anchor take the ground floor?	If yes, Alameda moves upstairs as a block sessional user and the whole model reruns on the anchor economics
2 Is the hair-restoration SPV pursued as a named anchor in its own right?	It has the best yield per sqm in the building and the lowest operational burden
3 Partner theatre now, modular unit later?	Recommended. It lets the anchor sell a full surgical offer with no capital, and the transfer agreement must be in place before they sign
4 Does Medbury Pharmaceuticals own the product shelf?	Worth NGN 47M to 99M a year and it needs to be written into the anchor agreement, not assumed
5 Keep reduced imaging for the Ring 2 specialists, or refer out?	Turns on how many Ring 2 rooms you expect to let. Releases 40 sqm and NGN 30M if dropped

On question 1 my view is yes. It earns more, it keeps the money in Nigeria, it gives the park the identity you are after, it matches Lagos, and it makes the Cloister and the private rentals easier to sell rather than harder. The cost is renegotiating a floor with Alameda that has not yet been contracted, which is the cheapest moment to do it.

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An exploration, not a recommendation to sign. Anchor revenue figures are illustrative and built from stated assumptions on room count, patients per day, case frequency and ticket, all of which should be tested against the Lagos pilot's actual trading data. Clinical tier definitions follow standard practice on anaesthesia and recovery but are not a substitute for a clinical governance review. Not a binding offer, and not legal or medical advice. Companions: the engagement note, the fit-out note and the rate assumptions.